

1. Administrative & Study Identification

Full Study Title: A Phase 2, Open-Label, Multicenter, Tumor-agnostic Study of MK-1084 as Monotherapy and in Combination With Cetuximab, in Participants With KRAS G12C-Mutant, Advanced Solid Tumors (KANDLELIT-014) **Brief Study Title:** A Clinical Study of MK-1084 in People With Advanced Solid Tumors (MK-1084-014) **Short Title/Acronym:** KANDLELIT-014 **Protocol Number:** MK-1084-014 (1084-014) **NCT Number:** NCT07209111 **Sponsor Name:** Merck Sharp & Dohme LLC (Merck) **Investigational Product:** Calderasib (MK-1084) alone or in combination with Cetuximab **Phase of Development:** Phase 2 **Medical Monitor:** Merck Sharp & Dohme LLC Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To evaluate the Objective Response Rate (ORR), measuring how many participants have their cancer respond (get smaller or go away) to calderasib alone or in combination with cetuximab; to evaluate the safety of the treatments by tracking the number of participants who experience one or more Adverse Events (AEs); and to track the number of participants who discontinue treatment due to an AE. **Secondary Objectives:** To evaluate Progression-free Survival (PFS), Duration of Response (DOR), and Overall Survival (OS).

2.2 Background & Rationale Mutations in KRAS are among the most prevalent genetic mutations found in cancer, with the KRAS G12C mutation occurring with high frequency in various solid tumors. This study explores the potential of the investigational drug calderasib (a potent and specific KRAS G12C covalent inhibitor), used alone or together with the EGFR inhibitor cetuximab, in treating advanced solid tumors (malignant neoplasms). The goal is to understand if these targeted treatments can shrink or eliminate the cancer, offering vital new treatment options and potentially improving the care and quality of life for patients harboring this specific mutation.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Trial Status:** Not Yet Recruiting **Control Type:** Multiple Experimental Arms (Monotherapy and Combination therapy) **Blinding:** Open-label **Randomization:** Tumor-agnostic cohort assignments

3.2 Planned Enrollment & Duration Target **\$N\$:** 150 participants
Number of Sites: Multicenter (Global, including sites in the US, Australia, Denmark, Germany, Israel) **Estimated Study Start:** 04-Dec-2025 **Estimated Primary Completion:** 09-Apr-2032

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Age ≥ 18 years. 2. Has locally advanced unresectable or metastatic solid tumor malignancy (other than colorectal cancer) and has progressed on, or following, standard of care systemic treatment. 3. Has a tumor that demonstrates the presence of the Kirsten rat sarcoma (KRAS) G12C mutation.

4.2 Exclusion Criteria 1. Has uncontrolled, significant cardiovascular disease or cerebrovascular disease. 2. Has known additional malignancy that is progressing or has required active treatment within the past 3 years. 3. Has known active central nervous system metastases and/or carcinomatous meningitis and/or primary brain tumors. 4. Has an active infection (other than those permitted per protocol) requiring systemic therapy. 5. Has not adequately recovered from major surgery or has ongoing surgical complications.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: * **Calderasib (MK-1084):** Administered orally. Per protocol, treatment has no maximum number of cycles. * **Cetuximab (Trade Name: Erbitux; ATC Code: L01FE01):** Administered at 500 mg/m^2 via intravenous (IV) infusion once every 2 weeks (Q2W). *EMA Product Information: https://www.ema.europa.eu/en/documents/product-information/erbitux-epar-product-information_en.pdf*

Route of Administration: Oral (calderasib) and Intravenous (cetuximab). **Duration of Treatment:** Participants will be treated until they experience disease progression, unacceptable toxicity, or any other criteria for discontinuation are met.

5.2 Concomitant & Prohibited Therapy Permitted: Standard supportive care for advanced cancer. **Prohibited:** Concurrent investigational anti-cancer therapies; therapies with known severe drug-drug interactions with the investigational products.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
1	-28 to 0	Screening, Informed Consent, Medical History, Tumor Imaging, KRAS G12C testing
2	0 to 14	Baseline, Cohort Assignment, First Dose of MK-1084 $\pm$ Cetuximab

| | **Interim** | Q2W | Safety Labs, Cetuximab Infusion (if applicable), AE monitoring, Tumor Imaging (approx. Q6W-Q8W) | | **End of Study** | Upon Progression | Final Safety/Vital Signs, Exit Interview, Follow-up for Survival |

7. Outcome Measures (Endpoints)

7.1 Primary Endpoints * **Objective Response Rate (ORR)**: Defined as the percentage of participants who experience a Complete Response (CR) or Partial Response (PR). Evaluated using Response Evaluation Criteria In Solid Tumors (RECIST) Version 1.1 by Blinded Independent Central Review (BICR). * **Adverse Events (AEs)**: Number of participants who experience one or more AEs. * **Discontinuations**: Number of participants who discontinue study treatment due to an AE.

7.2 Secondary & Exploratory Endpoints * Progression-Free Survival (PFS) * Duration of Response (DOR) * Overall Survival (OS)

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Continuous monitoring of adverse effects (any unfavorable and unintended sign, symptom, disease, or worsening of preexisting condition) and unfavorable symptoms leading to treatment discontinuation, irrespective of causality to study treatment. **Serious Adverse Events (SAEs)**: Reported within standard 24-hour global pharmacovigilance timelines. **Data Monitoring Committee (DMC)**: Safety data reviewed periodically by an internal sponsor safety monitoring committee.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) for primary efficacy response endpoints; Safety Set for all participants receiving at least one dose of study medication. **Sample Size Justification**: Target $N = 150$ powered to assess the ORR across different tumor-agnostic cohorts. **Handling of Missing Data**: Missing RECIST evaluations will be censored at the last evaluable time point for time-to-event endpoints.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki. **Monitoring Plan**: Routine onsite and remote monitoring visits to ensure protocol adherence and source data verification. **Audit Trail**: eCRF logging, independent central review for RECIST 1.1 imaging, and data clarification mechanisms.

11. Study Identifiers & Governance

Primary ID: MK-1084-014 **Posting ID:** 1117907791746329990 **Registry Number:** NCT07209111 **Sponsor/Lead Organization:** Merck Sharp & Dohme LLC (Merck) **Study Title:** KANDLELIT-014 **Official Regulatory Title:** A Phase 2, Open-Label, Multicenter, Tumor-agnostic Study of MK-1084 as Monotherapy and in Combination With Cetuximab, in Participants With KRAS G12C-Mutant, Advanced Solid Tumors

12. Clinical Framework (Oncology / KRAS G12C Specific)

Primary Condition: Advanced Solid Tumors **Disease Area / Preferred Name:** Neoplasm Malignant / Malignant Neoplasms (Preferred UMLS Name: Malignant Solid Tumor) **Semantic Type:** Neoplastic Process **Definition:** A neoplasm composed of atypical neoplastic, often pleomorphic cells that invade other tissues. Malignant neoplasms often metastasize to distant anatomic sites and may recur after excision. The most common malignant neoplasms are carcinomas, Hodgkin and non-Hodgkin lymphomas, leukemias, melanomas, and sarcomas. **Diagnosis Threshold:** KRAS G12C Mutation Positive **Medical Methodology:** Targeted Therapy (KRAS G12C Inhibitor & EGFR Inhibitor) **Optimization Target:** Objective Response Rate (Tumor Shrinkage per RECIST v1.1)

Ontology & Classification Codes: * **MeSH Code:** D009369 * **HPO Code:** HP:0002664 * **SNOMED ID:** 1240414004 * **SNOMED Hierarchy:** 108369006|Neoplasm|Neoplasm (morphologic abnormality), 400177003|Neoplasm and/or hamartoma|Neoplasm and/or hamartoma (morphologic abnormality), 416939005|Proliferative mass|Proliferative mass (morphologic abnormality), 4147007|Mass|Mass (morphologic abnormality), 52988006|Lesion|Lesion (morphologic abnormality), 49755003|Morphologically abnormal structure|Morphologically abnormal structure (morphologic abnormality), 118956008|Morphologically altered structure|Body structure, altered from its original anatomical structure (morphologic abnormality), 123037004|Body structure|Body structure (body structure), 30217000|Proliferation|Proliferation (morphologic abnormality), 57697001|Growth alteration|Growth alteration (morphologic abnormality)

13. Study Procedures & Methodology

Management Pathway: Systemic oncology clinical pathway **Education Protocol (HCP):** Site initiation visits, protocol training, and RECIST assessment criteria **Education Protocol (Patient):** Informed consent, oral drug self-administration logs, and toxicity reporting **Quality Audit Mechanism:** Investigator site monitoring & Independent Central Imaging Review

14. Observation & Follow-Up Schedule

Total Duration: Follow-up spanning to estimated study completion in 2032 **Onsite Visit Cadence:** Every 2 weeks (Q2W) to align with Cetuximab infusions (if in combination arm) **Remote Monitoring Frequency:** Routine sponsor intervals **Checklist Review Interval:** Radiographic imaging every 6-8 weeks

15. Sign-off & Approval

Role	Name	Signature	Date		:---	:---	:---	:---
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]					
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]					Lead
Statistician	[Name]	_____	[DD-MMM-YYYY]					